Clinical Guideline for:
Women declining blood or blood products in pregnancy, labour or postnatally

Summary

This guideline explains the management of pregnant women who decline to have blood products administered.

Declining blood products may be due to personal concerns as well as religious beliefs. It is important to develop a plan with women during pregnancy in order to establish which products would be acceptable to them in an emergency situation.

Key Points

The essential elements of this guideline are:

- If a woman is deemed to have the capacity to make decisions, her informed decisions must be respected even if this results in her death. No-one else is able to make decisions on her behalf.
- When a woman who declines blood products develops an obstetric hemorrhage, she has a 44-fold increased risk of dying because of her choice to decline transfusions.
- To administer blood to a woman who has steadfastly refused to accept it either by provision of an Advanced Medical Directive or by its exclusion in a consent form, is unlawful, ethically unacceptable and may lead to criminal and/or civil proceedings
- The woman will be advised to deliver in a Consultant led unit with access to emergency services if required.
- The on-call Consultant Obstetrician and Anaesthetist for Labour ward must be informed when any woman who may decline blood products is admitted
- Ensure there is a signed copy of an Advanced Decision or Advance Directive or Blood component refusal consent form. If not this must be completed and a copy placed in the electronic patient record.
- Women undergoing operative treatment must sign the procedure consent – blood component refusal form in the electronic patient record.

CONTENTS

Section		Page
1	Introduction	3
2	Antenatal care	3
3	Management in labour	4
4	Significant haemorrhage	4
5	Postnatal	5
6	Legal position	5
7	Auditable standards	5
8	References	6
9	Publication details	7
Appendices		
Appendix 1	Adding a blood refusal form to the patient record	8

1.0 INTRODUCTION

1.1 Declining blood products may be due to personal concerns as well as religious beliefs. If a woman is a Jehovah's Witness please also refer to RDUH guideline on Patients Declining a Blood Transfusion.

1.2 For many healthy women, the first time the subject of accepting blood or blood products is encountered will be during their initial antenatal visit. It is important that women are asked at this appointment to establish if they may choose to decline blood products in an emergency situation.

It is important that women who decline blood products are aware that a pregnant woman faces a 5% risk of sustaining a significant haemorrhage during the course of her gestation. Bleeding may occur antenatally, as in the case of placenta praevia or abruption, or following delivery from uterine atony or genital tract lacerations. When a woman who refuses blood products develops an obstetric hemorrhage, she has a 44-fold increased risk of dying because of her choice to decline to transfusions.

1.3 To administer blood to a woman who has steadfastly refused to accept it either by provision of an Advanced Medical Directive or by its exclusion in a consent form, is unlawful, ethically unacceptable and may lead to criminal and/or civil proceedings.

1.4 Blood products include:

- Anti D (See Clinical Guideline for the management of antenatal women who are Rh D negative or have atypical red cell antibodies)
- Fresh Frozen Plasma
- Platelets

2.0 ANTENATAL CARE

2.1 **As soon as it is known** that a woman may refuse blood products during pregnancy, labour or the puerperium she must be referred to a Consultant Obstetrician. This should usually take place following the booking appointment. A plan of care must be discussed and agreed and clearly documented in the woman's electronic patient record.

2.2 The woman must be given all the relevant information including the increased risk of hysterectomy should haemorrhage occur.

2.3 The woman will be advised to deliver in a Consultant led unit with access to emergency services if required.

2.4 The woman's wishes must be clearly documented in the electronic patient record. This must include a list of treatments which she has consented to in writing.

2.5 A plan for management must be available should haemorrhage occur.

2.6 A copy of an 'Advanced Decision' **OR** a completed 'Blood Refusal Form' must be present in the electronic patient record. (See Appendix 1)

2.7 Hb should be optimised before delivery to prevent avoidable anaemia and haematinics should be recommended if the Hb is less than 10.5 g/dl.

3.0 MANAGEMENT IN LABOUR

3.1 The on-call Consultant Obstetrician and Anaesthetist for labour ward must be informed when any woman who may decline blood products is admitted. Out of hours the SpR would inform the on-call Consultant Obstetrician if necessary.

3.2 Ensure there is a signed copy of an Advanced Decision or Advance Directive or the blood refusal consent form. If not this must be completed in the electronic patient record.

3.3 The pharmacological, mechanical and surgical procedures to avert the use of banked blood and blood components should be considered at an early stage.

3.4 All such women should be advised to have active management of the third stage of labour.

3.5 The woman's right to refuse treatment must always be respected.

3.6 Women should always be given the opportunity to change their mind about the use of blood products.

3.7 Full blood count, group & save, platelet and clotting studies must be taken on admission.

3.8 If required, perineal repair should be undertaken immediately.

3.9 If Caesarean section is required it should be carried out by a Consultant Obstetrician.

3.10 Consider the use of cell salvage if this is acceptable to the woman. (See Clinical Guideline for use of intraoperative cell salvage (ICS) in obstetrics).

3.11 Advice should be taken from the Consultant Haematologist before considering the use of heparin to combat disseminated intravascular coagulation.

3.12 Hysterectomy is usually the last resort in the treatment of obstetric haemorrhage but delay will increase risk. In these cases a timely hysterectomy may save the woman's life.

3.13 When a hysterectomy is performed the uterine arteries should be clamped as early as possible in the procedure. Sub-total hysterectomy can be as effective as total hysterectomy, as well as being quicker and safer. In some cases there may be a place for internal iliac artery ligation.

3.14 Women undergoing operative treatment must sign the Blood Refusal Form (see Appendix 1).

4.0 SIGNIFICANT HAEMORRHAGE

4.1 This should be managed as any other ante- or post-partum haemorrhage (See Obstetric Haemorrhage Clinical Guideline).

4.2 Consider the use of factor V Leiden in discussion with Consultant Haematologist.

4.3 Expert advice will be available from the on-call consultant Haematologist.

4.4 **Post haemorrhage**
For severe anaemia give oxygen and use Erythropoietin 300 U/kg x 3 per week subcutaneously, without delay. This shortens lag period of erythropoiesis and accelerates haemoglobin recovery. Dosage for renal anaemia (50U/kg) is ineffective for severe blood loss anaemia.

4.5 Consider elective ventilation on intensive care unit.

4.6 Iron supplementation is essential. The use of IV iron in consultation with Pharmacy is recommended in preference to oral iron. Augment with vitamin B-12 and folic acid.

5.0 POSTNATAL

5.1 The woman should be encouraged to report promptly if she has any concerns about bleeding during the puerperium.

5.2 If needed, an epidural blood patch is not prohibited on religious grounds.

6.0 LEGAL POSITION

6.1 Legally any adult patient (18 years or over), who has the necessary mental capacity to do so is entitled to refuse treatment, even if refusal will result in their death. No other person is legally able to consent to treatment for that adult or to refuse treatment on that person's behalf.

6.2 If standard treatment is not controlling the bleeding the woman should be informed that a blood transfusion is strongly recommended. Any woman is entitled to change her mind about a previously agreed treatment plan, but this change must be witnessed and a contemporaneous entry be made in the woman's electronic patient record. The doctor must be sure that the woman's decision is her own and she is not being subjected to pressure from others.

6.3 If the woman's condition deteriorates, her most recent decision must be upheld and cannot be over ruled by anyone.

6.4 In an emergency where the woman is unable to communicate or lacks mental capacity and in the absence of an Advanced Decision or Advanced Directive, the obstetrician

must act in the best interests of the woman. In the case of mental capacity, an assessment must be completed in accordance with the Mental Capacity Act.

6.5 Such relatives or associates may be invited to provide evidence of the woman's status in the form of an applicable advanced directive document completed by the woman and the signature witnessed.

7.0 AUDITABLE STANDARDS

7.1 All women who choose to decline blood products in the antenatal, intrapartum or postnatal period should have a plan of care that has been made in conjunction with the woman involving the Consultant Obstetrician, Consultant Anaesthetist and the Haematologist if necessary.

8.0 REFERENCES

Buscotti, D. Copplestone, A. (1995) *Management of Blood loss in Jehovah's Witnesses* British Medical Journal 311 pp1115-1116

Mann, C.M. et al (1992) *Management of Severely Anaemic Patient who Refuses Transfusion: Lessons Learned during the Care of a Jehovah's Witness* Annals of International Medicine 117 pp1042-104

Royal College of Surgeons (1996) *Code of Practice for the Surgical Management of Jehovah's Witnesses* Royal College of Surgeons of England London

Management Of Anaesthesia for Jehovah's Witnesses (2nd Ed) The Association of Anaesthetists of Great Britain and Ireland Publication. November 2005

Saving Mother's Lives: reviewing maternal deaths to make motherhood safer: 2006-2008. The Eigth Report on the Confidential Enquiries into Maternal Deaths in the United Kingdom. CMACE. London.

Thomas J.M. (1994) *The World wide need for education in non blood management in obstetrics and gynaecology.* Journal of the Society of Obstetricians and Gynaecologists of Canada. 16:1483-7

9.0 PUBLICATION DETAILS

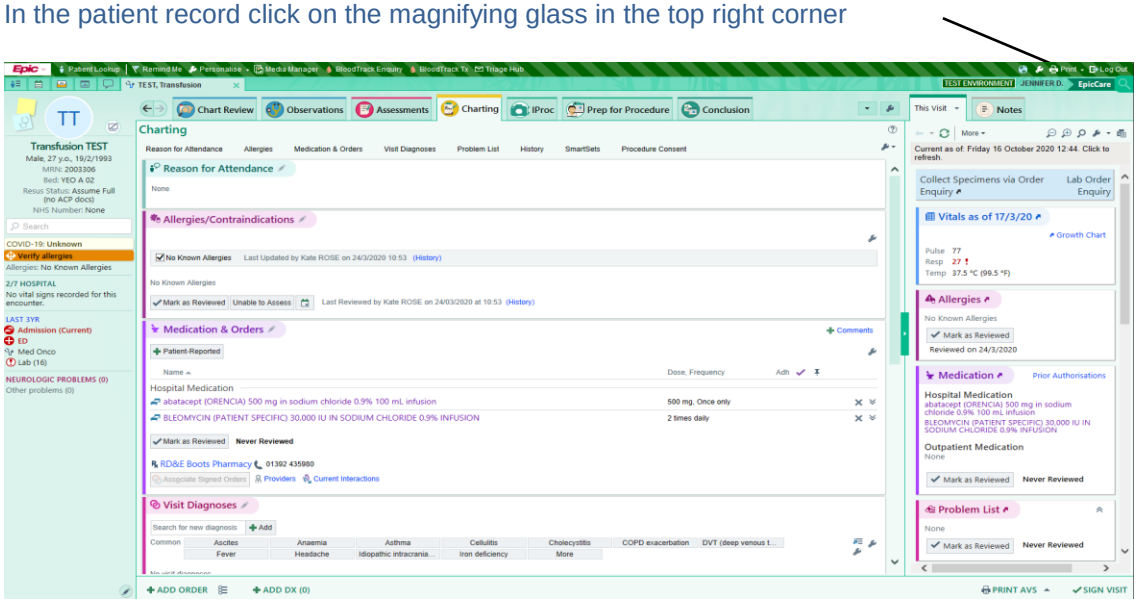
Author of Clinical Guideline	Senior Maternity Matron – Hospital
Directorate/Department responsible for Clinical Guideline	Specialist Services/ CWH/Maternity
Contact details	Ext. 6682
Version number	3.1
Replaces version number	3.0
Date written	August 2008
Approving body and date approved	Maternity Guideline Group 11/12/2024
Review date	11/06/2029
Expiry date	11/12/2029
Date document becomes live	25/03/2026

APPENDIX 1

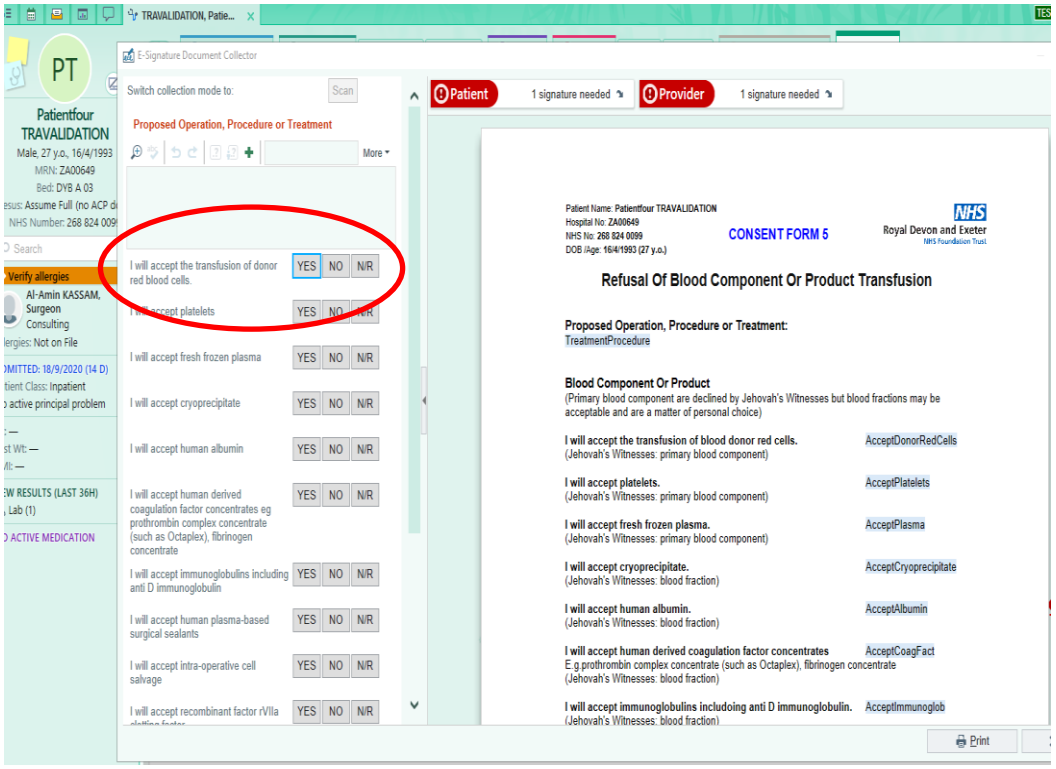
Blood Transfusion – Adding a Blood Refusal Form to the Patient Record

Blood refusal forms can be added to the patient record, in accordance with the Policy for Patients who Refuse Blood

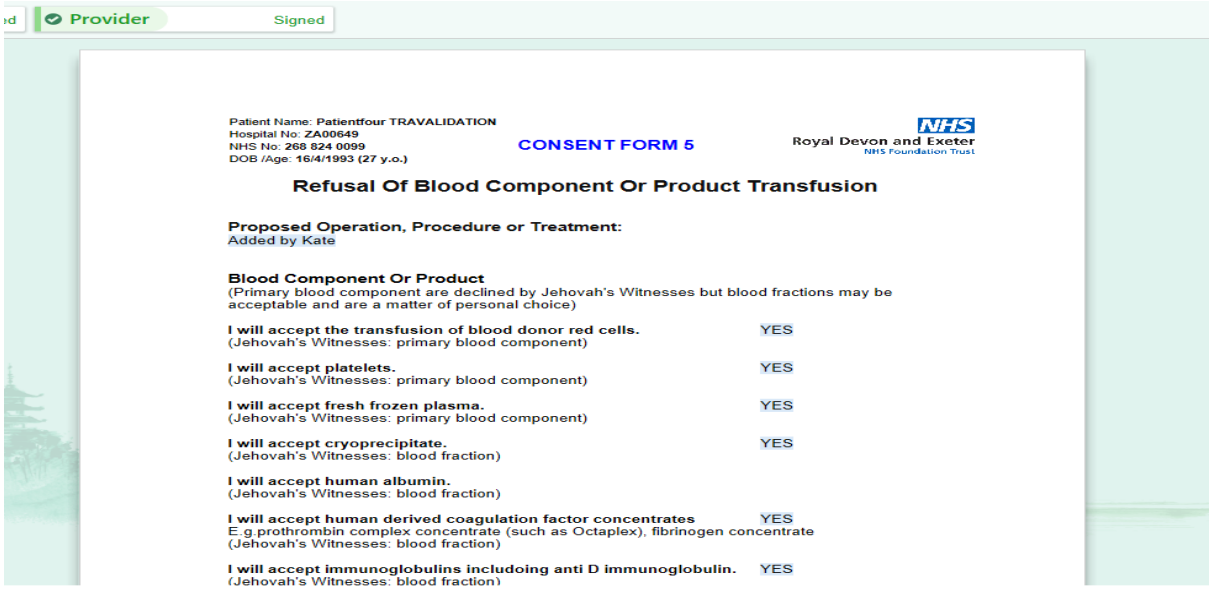
In the patient record click on the magnifying glass in the top right corner



Type 'document list' in the search field, select 'new document type' and type 'refusal' into the field. The refusal access field will then appear on the screen, select 'E-Sign' and then the form will appear on the screen. On the refusal form select the type of components and products that the patient will/or will not accept by clicking the 'YES' or 'NO' buttons, use NR if the patient is unable to confirm if the product can be given or not.



The refusal form (on the right hand side) will populate with the selected responses


Provider Signed

Patient Name: Patientfour TRAVALIDATION
Hospital No: ZA00649
NHS No: 268 824 0099
DOB /Age: 16/4/1993 (27 y.o.)

CONSENT FORM 5

NHS Royal Devon and Exeter NHS Foundation Trust

Refusal Of Blood Component Or Product Transfusion

Proposed Operation, Procedure or Treatment:
Added by Kate

Blood Component Or Product
(Primary blood component are declined by Jehovah's Witnesses but blood fractions may be acceptable and are a matter of personal choice)

I will accept the transfusion of blood donor red cells. YES
(Jehovah's Witnesses: primary blood component)

I will accept platelets. YES
(Jehovah's Witnesses: primary blood component)

I will accept fresh frozen plasma. YES
(Jehovah's Witnesses: primary blood component)

I will accept cryoprecipitate. YES
(Jehovah's Witnesses: blood fraction)

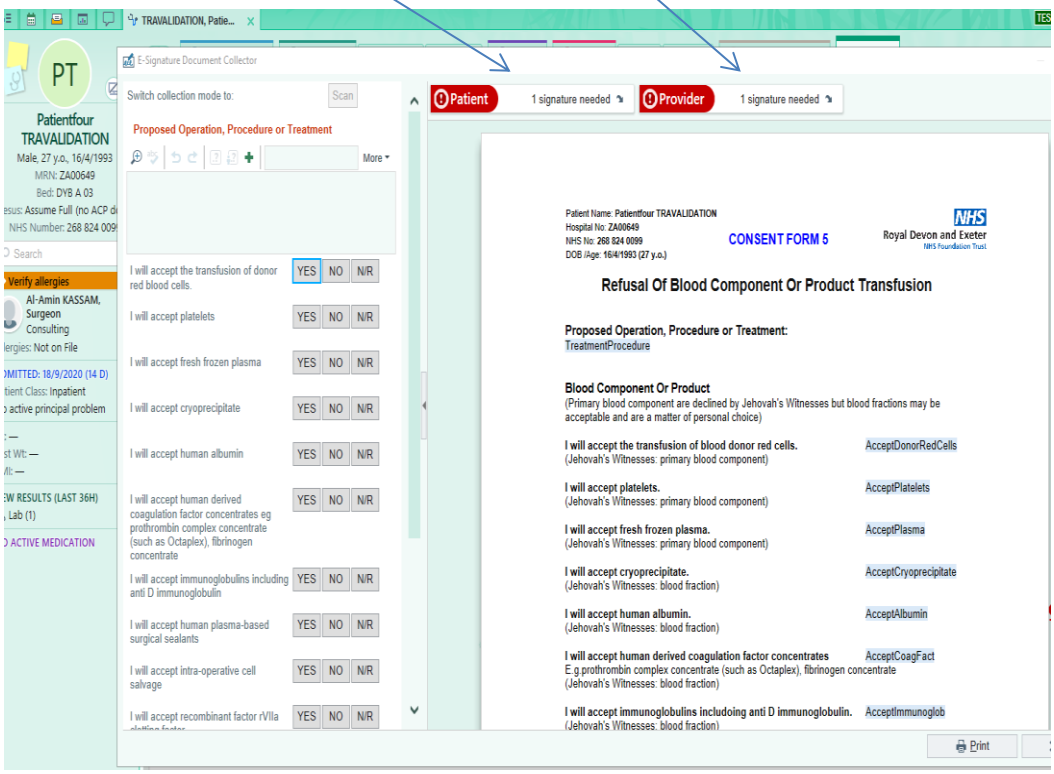
I will accept human albumin.
(Jehovah's Witnesses: blood fraction)

I will accept human derived coagulation factor concentrates YES
E.g.prothrombin complex concentrate (such as Octaplex), fibrinogen concentrate
(Jehovah's Witnesses: blood fraction)

I will accept immunoglobulins includoing anti D immunoglobulin. YES
(Jehovah's Witnesses: blood fraction)

This form than then be signed using the e-signature process, or it can be printed out to be signed and then scanned to the patient record.

Complete the patient and provider signature areas and press 'accept' to complete the process


TRAVALIDATION, Patie...

PT
Patientfour TRAVALIDATION
Male, 27 y.o., 16/4/1993
MRN: ZA00649
Bed: DYB A 03
NHS Number: 268 824 0099

Verify allergies
Al-Amin KASSAM, Surgeon
Consulting

E-Signature Document Collector

Switch collection mode to: Scan

Proposed Operation, Procedure or Treatment

Patient 1 signature needed
Provider 1 signature needed

I will accept the transfusion of donor red blood cells. YES NO N/R
I will accept platelets YES NO N/R
I will accept fresh frozen plasma YES NO N/R
I will accept cryoprecipitate YES NO N/R
I will accept human albumin YES NO N/R
I will accept human derived coagulation factor concentrates eg prothrombin complex concentrate (such as Octaplex), fibrinogen concentrate YES NO N/R
I will accept immunoglobulins including anti D immunoglobulin YES NO N/R
I will accept human plasma-based surgical sealants YES NO N/R
I will accept intra-operative cell salvage YES NO N/R
I will accept recombinant factor rVIIa YES NO N/R

Patient Name: Patientfour TRAVALIDATION
Hospital No: ZA00649
NHS No: 268 824 0099
DOB /Age: 16/4/1993 (27 y.o.)

CONSENT FORM 5

NHS Royal Devon and Exeter NHS Foundation Trust

Refusal Of Blood Component Or Product Transfusion

Proposed Operation, Procedure or Treatment:
TreatmentProcedure

Blood Component Or Product
(Primary blood component are declined by Jehovah's Witnesses but blood fractions may be acceptable and are a matter of personal choice)

I will accept the transfusion of blood donor red cells. AcceptDonorRedCells
(Jehovah's Witnesses: primary blood component)

I will accept platelets. AcceptPlatelets
(Jehovah's Witnesses: primary blood component)

I will accept fresh frozen plasma. AcceptPlasma
(Jehovah's Witnesses: primary blood component)

I will accept cryoprecipitate. AcceptCryoprecipitate
(Jehovah's Witnesses: blood fraction)

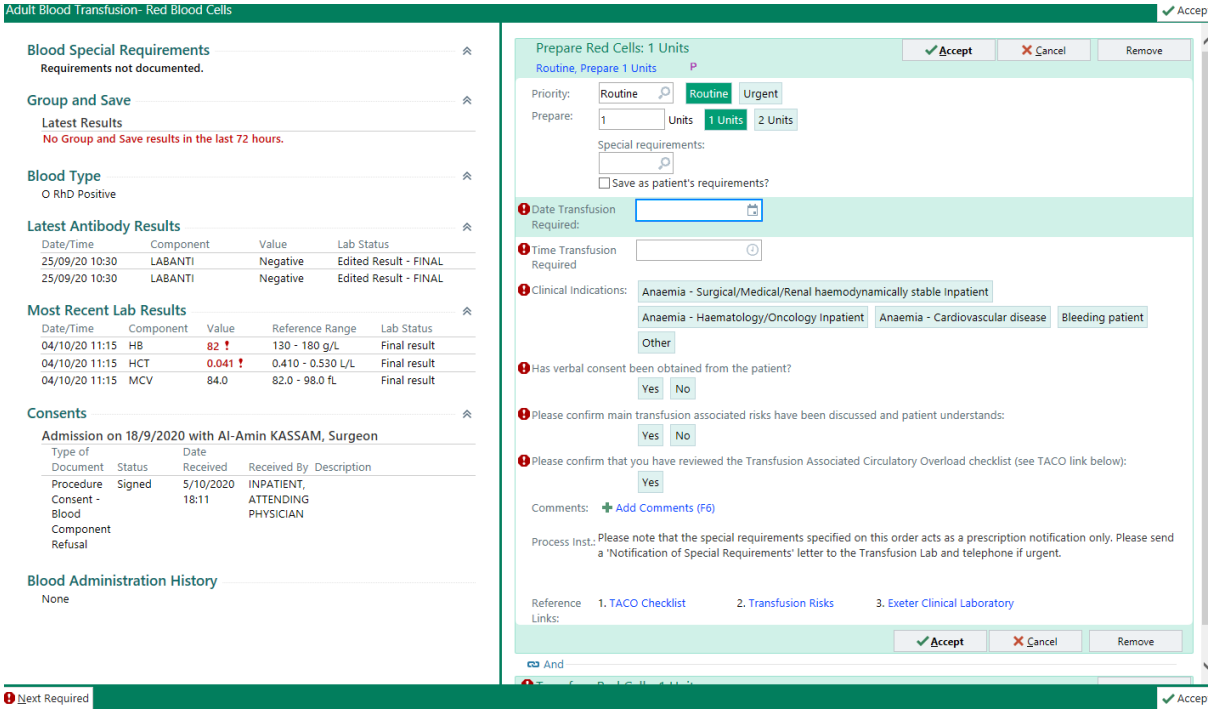
I will accept human albumin. AcceptAlbumin
(Jehovah's Witnesses: blood fraction)

I will accept human derived coagulation factor concentrates AcceptCoagFact
E.g.prothrombin complex concentrate (such as Octaplex), fibrinogen concentrate
(Jehovah's Witnesses: blood fraction)

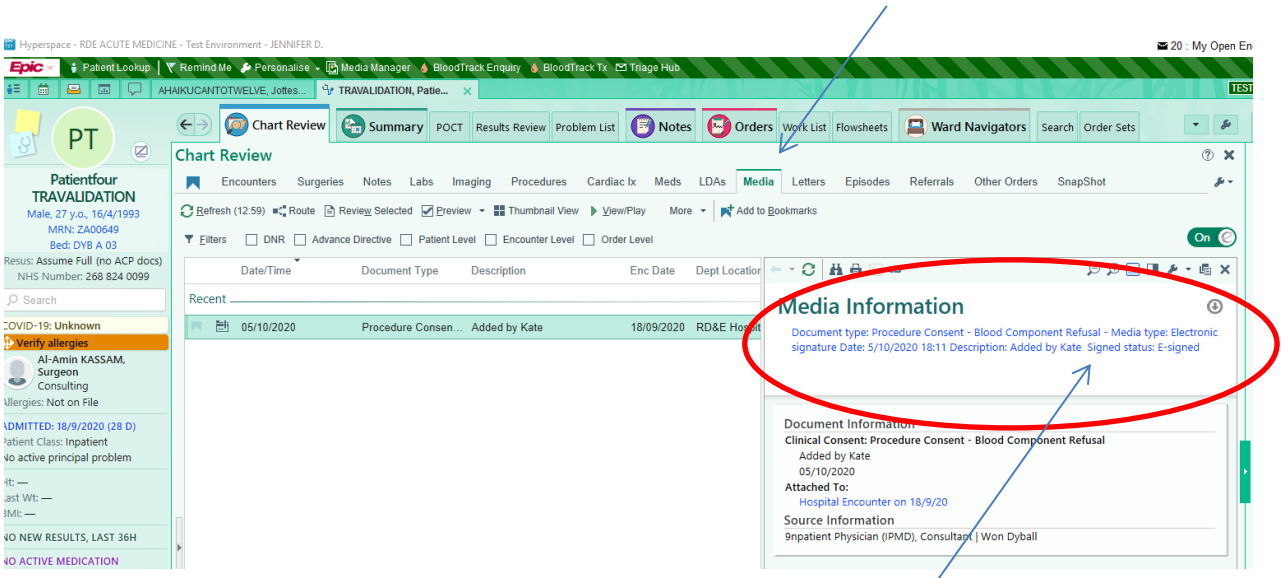
I will accept immunoglobulins includoing anti D immunoglobulin. AcceptImmunoglob
(Jehovah's Witnesses: blood fraction)

Print

Once the form has been complete it will be displayed in any blood order screens, with a hyperlink to the signed form Note: the presence of the form does not prevent you from completing the blood order, it is there to guide on blood ordering



The form can also be accessed in the chart review tab, under media



A hyperlink is available to access the form and view the contents